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Socioeconomic status and access to psychotherapy

Helen Niemeyer  | Christine Knaevelsrud

Department of Clinical Psychological Intervention, Freie Universität Berlin, Berlin, Germany

Correspondence

Helen Niemeyer, Department of Clinical Psychological Intervention, Freie Universität Berlin, Schwendenerstr. 27, 14195 Berlin, Germany.
Email: Helen.niemeyer@fu-berlin.de

Abstract

Objective: Individuals with low socioeconomic status (SES) have significantly higher prevalence rates of mental disorders than those with higher SES, but are less likely to receive treatment. Studies from the United States show that individuals with higher SES are more likely to be offered a therapy place than those with low SES. In Germany, where the public health care system is without financial barriers, studies on access to treatment are lacking.

Method: The present study investigated whether the SES of persons seeking psychotherapeutic help in Germany influences psychotherapists' decision to provide access to treatment. For this purpose, 504 licensed outpatient psychological psychotherapists participated in an online study, in which they read a series of case vignettes and rated how likely they would be to offer treatment in each case. SES was operationalized by occupation and language use. Additionally, we investigated whether treatment offers were affected by the disorder and the gender of the help seeker, the therapists' socioeconomic background, and other characteristics in interaction with SES. Bonferroni-corrected multifactorial repeated measures analysis of variances (ANOVAs) and mixed ANOVAs were calculated.

Results: Help seekers with low SES were 4.1% less likely to be offered treatment than those with moderate to high SES ($F(1, 503) = 115.64; p = 0.000; \eta^2 = 0.187$). The influence of SES on the probability of a therapy place offer varied

significantly depending on the patient's disorder ($p = 0.000$): SES had a greater effect for bulimia than for depression and social anxiety disorder. Overall, the effect was small ($\eta^2 = 0.026$). No significant interaction between the gender of potential clients and SES was found ($p = 0.062$). The influence of SES on treatment accessibility varied depending on the therapeutic method for which the therapist was licensed and on the social status of the district in which the therapist's practice was located. The therapist's socioeconomic background (parental SES) did not interact with the potential client's SES.

Conclusion: Therapist biases contributing to unequal access to treatment for individuals with low SES and other characteristics can aggravate mental health care disparities. Even though the effects we found were relatively modest, awareness of the association between SES and offers of therapy places might help practitioners to reflect on their own selection practices.

KEYWORDS

psychotherapy, socioeconomic status

1 | INTRODUCTION

Socioeconomic status (SES) describes an individual's position in the economic hierarchy of a society that is characterized by social inequality (Lampert & Kroll, 2006). It is usually measured as a combined index of education, income, and occupation (Lampert et al., 2013). The impact of the economic positioning on social and psychological experiences is wide-ranging and multifaceted (Kraus et al., 2017; Niemeyer et al., 2019), and economic inequality especially impacts upon physical and mental health (Kraus et al., 2017; Robert Koch-Institut, 2015). Several international studies indicate that the prevalence of psychological disorders is higher in persons with low SES than in those with high SES (Delgadillo et al., 2016; Lorant, 2003; Pulkki-Råback et al., 2011; Silva et al., 2016). For instance, in a large German sample representative of the adult population, strikingly higher 12-month prevalence rates for the presence of any mental disorder were observed for women and men with low SES (43.3% and 32.3%, respectively) than for those with high SES (27.4% and 17.7%; Jacobi et al., 2014).

Yet, individuals with low SES are significantly less often in outpatient psychotherapeutic treatment, as studies from Britain, Canada, the United States, and Germany show (Delgadillo et al., 2016, 2018; Dhingra et al., 2010; Epping et al., 2017; Ngamini Ngui et al., 2012). A study analyzing data from a large public health insurance company in Germany revealed social disparities in the utilization of treatment, disadvantaging both women and men with low SES despite the fact that there are no financial barriers to treatment in Germany (Epping et al., 2017).

According to Krupnick and Melnikoff (2012), practical obstacles such as difficulties in accessing the therapy location due, for instance, to childcare or work commitments, might make it difficult for people with low SES to seek psychotherapy. Further potential obstacles to seeing a therapist include distrust of authorities or the perception of

social differences between patient and therapist. Moreover, health literacy, defined as a high degree of knowledge about mental disorders and treatment options, goes hand in hand with more pronounced and adequate help-seeking behavior, and individuals with low SES often have lower health literacy (Holman, 2015; Rüscher et al., 2011; Schomerus & Angermeyer, 2008; Thornicroft, 2008; Wright et al., 2007).

Individuals with higher SES are also more likely to be recommended psychotherapy than individuals with low SES (Garb, 1997), and therapists ascribe a poor prognosis to clients with low SES or have lower expectations of treatment success (Garb, 1997; Stein et al., 1972; Sutton & Kessler, 1986). In a case vignette study, Smith et al. (2011) examined the influence of a potential client's social class on the first therapeutic impression of psychological counselors in training. The authors found that the expectations were more negative when the client was presented as a member of the working class as compared to the middle or high social classes. The therapeutic relationship and collaboration with a potential working-class client was rated as less in-depth, effective and enjoyable (Smith et al., 2011). Davis and Proctor (1989) found that therapists see people with low incomes as resistant, suspicious, apathetic and passive, and believe that they cannot articulate themselves well. Moreover, therapists expected low-income clients' values to be incompatible with psychotherapy and believed that they would be unable to benefit from therapeutic treatment.

Recent studies from the United States explicitly considered selection processes in therapy placement. In the first study to investigate a potential SES-related selection, using written case vignettes, Thompson et al. (2014) found no influence of SES on therapists' willingness to work with a client. In a field study conducted 2 years later by Kugelmass (2016), hypothetical working-class and middle-class help seekers (voiced by actors) left voicemail messages for 320 New York psychotherapists, with social class operationalized based on language use. Middle-class help seekers were three times more likely to be offered an appointment than working class people. In another, recent field study with simulated e-mails, Kugelmass (2018) concluded that the contacted psychotherapists preferred potential clients with a higher education over less educated clients. Taken together, while the findings vary, most studies indicate an influence of SES on therapists' expectations and treatment offers.

Studies from Germany in this regard are scarce. The health care system in Germany can generally be assessed as very good in international comparison (Strauß, 2015). Outpatient psychotherapy has been one of the compulsory benefits of the statutory health insurance companies since 1967 (Epping et al., 2017). About 90% of the German population have statutory health insurance, and thus there are no financial barriers for the majority regarding access to psychotherapy (Epping et al., 2017).

Despite the overall good situation in Germany, a report published in 2018 by the Federal Chamber of Psychotherapists (BPTK) on the status of outpatient psychotherapeutic care showed that the demand for psychotherapy significantly exceeds the available supply. While there is capacity for an average of 14 new resident therapy placements over a 6-month period, an average of 23 patients inquire after a therapy place every month (BPTK Federal Chamber of Psychotherapists/Bundespsychotherapeutenkammer, 2018). The waiting time to start psychotherapy is on average 19.9 weeks (BPTK Federal Chamber of Psychotherapists/Bundespsychotherapeutenkammer, 2018). This undersupply leads to a situation in which therapists must refuse many potential clients, and therapists have a high level of professional autonomy regarding which clients they accept.

Furthermore, therapists have a tendency to locate their practices in socially stronger urban districts, and the number of therapy practices in districts classified as socially strong is many times higher than that in districts classified as socially weak (e.g., Joint State Committee for Berlin Gemeinsames Landesgremium für Berlin, 2018). The therapist's own socioeconomic background may also be important regarding the effect of a client's SES on the offer of treatment. Positive intergroup contacts can result in an engagement of the beneficiary group for the interests of the disadvantaged group (Reimer et al., 2017), and the influence of clients' SES on treatment offers might be lower for therapists with a lower socioeconomic background.

A number of studies have investigated the accuracy of perceiving social class based on signals such as behavior, speech, or physical appearance (Kraus et al., 2017). That is, the social class of others can be perceived rapidly and accurately based on a small amount of information such as language use (e.g., Labov, 2006). Such information

influences judgments by activating stereotypes (e.g., Fiske et al., 2002). The adaptive strategy of using stereotypes to reduce the complexity of the world by forming categories and assigning equal properties to all members of a category is common to everybody (Hamilton, 1981; van Ryn & Burke, 2000). Therefore, it is reasonable to assume that therapists also have such nonclinical biases in the perception of help seekers with low SES, which affect their consequent decisions to offer access to treatment.

Beyond SES, studies indicate that health care providers have reservations towards certain disorders. In a representative survey of 1042 psychological and medical psychotherapists and psychiatrists in Germany, Zepf et al. (2003) found that patients with pain disorders, addictive disorders, anorexia nervosa, and psychosomatic, personality, and psychotic disorders were often not accepted for treatment despite an assumed need for such treatment. Another survey encompassing 229 psychological psychotherapists in East Germany (Behrendt et al., 2014) suggested resentments towards substance-related disorders, and Strauß (2015) likewise indicated that patients with personality disorders have a harder time finding a therapy place. Taken together, these findings indicate that the disorder itself might also affect the decision to offer access to treatment. Other studies found that psychotherapists preferred female over male clients (Kugelmass, 2016; Zivian et al., 1992). Accordingly, it is conceivable that the influence of a client's SES on the offer of treatment varies depending on the disorder and the gender of the client. The presence of certain preferred characteristics might lead to less attention being paid to other characteristics and vice versa. In this way, SES may have a weaker or stronger influence if a potential client has other preferred (or non-preferred) characteristics.

Moreover, there might be a relationship between a person's SES and the expected fit with a therapeutic procedure. Garb (1997) found that individuals with low SES were more likely to receive a recommendation for supportive therapy and individuals with higher SES were recommended an insight-oriented therapy. In a study by Sutton and Kessler (1986), psychodynamic therapy was selected less often as suitable for patients with low SES. Moreover, the proportion of privately insured clients and self-payers in psychoanalysis is many times higher than of statutory insured clients (Zepf et al., 2003).

1.1 | Objectives

The aim of the current study was to investigate whether help seekers with low SES are less likely to be offered a therapy place than potential clients with middle to high SES by outpatient psychological psychotherapists in Germany. As the decision to offer a therapy place might also depend on the potential client's disorder, we further investigated effects of the disorder and interactions with SES. The three disorders depression, bulimia, and social phobia were chosen because they are relatively common and because the literature does not reveal that these disorders are "unpopular" among therapists, meaning that potential interactions with SES were more likely to be detected. We also investigated the effect of additional client and therapist characteristics on the probability of offering a therapy place, such as the gender of the client, parental SES of the therapist, and whether the practice is located in a socially weaker or stronger district. Finally, we examined the influence of the therapeutic method, that is analytical psychotherapy, psychodynamic psychotherapy or behavioral therapy, which are all covered by the public health insurance companies in Germany. The study was conducted using case vignettes and designed as an online study.

2 | METHODS

A pilot study with psychology students was conducted, in which the case vignettes were designed, evaluated and revised, and the feasibility of the study design was tested. The main study was preregistered in the Open Science Framework (OSF; <https://osf.io/qbd6u>) and approved by the ethics committee of the Faculty of Education and Psychology at Freie Universität Berlin. Data and R code are also available in the OSF (<https://osf.io/z34s2/>).

2.1 | Case vignettes

The case vignettes were designed as contact forms that had been filled out by hypothetical help seekers. The potential clients had to provide information on age, gender, marital status, and occupation, followed by a description of their current symptoms. The independent variables with their respective factor levels were SES (low, medium/high), disorder (social anxiety disorder, bulimia, depression) and gender (male, female). These were varied in the case vignettes such that each combination of characteristics was present exactly once. This resulted in a total of twelve case vignettes (see Supporting Information: Appendix 1 for three case vignettes).

With regard to SES, we distinguished between two levels, low and medium/high, as an explicit separation of medium and high SES was not in the focus of the investigation and because we expected that some therapists might also have concerns about clients with high SES. We operationalized SES by the stated occupation and indirectly by a specific use of language. Occupation was selected as one of the three components of SES as we considered it rather unrealistic for a potential client to provide information on educational qualifications or income at the first point of contact. For medium to high SES, we chose occupations that require a higher education, such as engineer, pharmacist, or orthopedic surgeon. For low SES, we chose occupations that do not require a higher education, such as cashier, florist, or cleaning staff. In addition, SES was operationalized by the use of language in the symptom description in the contact forms. The use of different language variations, so-called sociolects, makes it possible to recognize differences in SES within interpersonal contact (Kaesler, 2005; Niepold, 1981; Oerter, 1970; Schütte, 2013). Specifically, shorter sentences, simpler words, and proverbs or idioms were used to indicate low SES, whereas more complex sentences, more demanding words and formulations, and a greater number of adverbs were used as indicators of medium/high SES.

With regard to the disorders described in the case vignettes, we chose depression, social anxiety disorder, and bulimia, because previous research indicated no pronounced reservations of therapists towards these disorders. This should prevent any overly negative attitudes towards certain disorders, which might influence the response behavior too strongly. Additionally, we avoided disorders that might be associated with a time expenditure that goes beyond a normal therapy session, such as phobias due to in vivo exposure in behavioral therapy.

To maximize the authenticity of the hypothetical clients' symptom descriptions, we drew on diagnostic telephone interviews from another study, which were conducted in the Clinical-Psychological Intervention Department at the Freie Universität Berlin with treatment-seeking patients of a public health insurance company. In line with the recommendations of Evans et al. (2015), care was taken to ensure that the case vignettes did not differ in the number and severity of the symptoms presented, and in the length of the text.

The effect of age on offers of therapy was not a focus of the current study; therefore, age was kept in the range of 35–49 years across the vignettes. The frequency of the individual marital status information (single, married, divorced, and widowed) in the case vignettes resembled the distribution in this age group in the German population (Statistische Ämter des Bundes und der Länder, 2014). Accordingly, seven of the potential clients were married, three were single, and two were divorced. We did not include the category of widowed due to the comparatively very low frequency in this age group. The respective information was distributed relatively evenly across the case vignettes with respect to SES, the disorders, and gender.

2.2 | Therapist sample

Based on an a priori power analysis for an expected small effect of $f = 0.1$ for the main objective, we aimed to reach a sample of at least $N = 390$. The therapists eligible to participate in the study were identified on the online platforms www.therapie.de and [https://psych-info.de](http://psych-info.de). The recruitment was restricted to large cities to enable an investigation of potential differences between practices located in socially strong and weak urban districts, and to reduce variation due to possible differences in therapy availability between urban and rural regions. We endeavored

to recruit therapists in cities with marked differences between socially strong and weak districts. The cities with the highest rental prices in Germany (e.g., Munich, Frankfurt am Main; Empirica, 2019) were excluded so that sufficiently large proportions of the population with low, medium and high SES were present as potential clients in the city in which the therapists' practices were located.

The study took place in June and July 2019. No expense allowance was granted for participating in the study. However, to increase motivation to participate, all participants were entered into a lottery to win one of two Amazon vouchers worth €25 each.

2.3 | Measures

2.3.1 | Probability of a therapy place offer

The probability of offering a therapy place was the main outcome. To measure this, after reading each case vignette, the participants were asked the following question: "What is the probability that you would offer this person a therapy place if you had to make an intuitive decision based solely on the information available?" The question was answered on a 11-point Likert scale from 0% to 100%. We chose a percentage-based rating as this was judged to be easy for the participants to complete.

2.3.2 | Socioeconomic background of the psychotherapist

The socioeconomic background of the therapists was determined from the information provided by the participants on the parental SES. Educational attainment (lower-track secondary school, medium-track secondary school, higher-track secondary school/general university entrance qualification (Abitur), apprenticeship, and university/university of applied science) and occupation according to the International Socioeconomic Index of Occupational Status (Ganzeboom et al., 1992) were assigned a point value between 1 and 7 and combined into one index following the procedure by Lampert et al. (2013). The quintiles of the distribution of all index values were determined, and the middle 60% (second to fourth quintile) were categorized as medium socioeconomic background. For each participant, the higher value from the two index values of the parents was selected instead of taking a mean value (Lampert et al., 2013).

We also asked about the therapists' age and gender, the therapy method for which they had acquired their license to practice psychotherapy, whether they had billing authorization with the statutory health insurance ("Kassensitz"), and the location of their practice (city and district).

2.3.3 | Social status of the city district in which the therapist's practice is located

The Social Index from the Social Structure Atlas Berlin 2013 (Senate Department for Health and Social Affairs [Senatsverwaltung für Gesundheit und Soziales], 2013) was used to determine the social status of the districts in Berlin. The Social Index is a measure of the distribution of social burdens, including a number of indicators such as unemployment rate, the proportion of welfare recipients, and the proportion of those under 15 years of age living in a household with a minimum income. For Bremen, Hamburg, Cologne, and Leipzig, no social index exists, but the information on the single indicators was available and the classification into socially weak, medium, and strong districts was carried out by the authors. Supporting Information: Appendix 2 provides an overview of the classification of the city districts and the sources of information.

2.4 | Online study design

The study was conducted online using the survey software Unipark (EFS Survey, 2017) by Questback GmbH (2017). Participants reached the landing page of the survey by clicking on a link provided in the email inviting them to participate. They were told that the study was about how decisions on the allocation of therapy places are made. To prevent distortions in response behavior, they were not initially informed about the focus on SES. The participants were informed about the requirements for participating in the study, and after providing written informed consent, were asked to imagine that they had received the completed contact forms from potential new clients who were seeking treatment. It was further noted that the information in the contact form naturally provided a limited picture of the potential client and that an initial consultation and subsequent trial sessions would provide a much more comprehensive impression of each individual's concerns. Moreover, it was stated that due to their therapeutic experience with other clients, the participants probably already had a reliable feeling regarding whether they would be able and willing to work with a potential client. The participants were encouraged to consider the real context of their everyday work, in which they would certainly not be able to offer a place to all help seekers due to time constraints.

We refrained from specifying a number of vacant therapy places to prevent the therapists' decisions from being influenced by the number of clients that had already been accepted or rejected. To rule out a possible effect of potential clients' time availability on the participants' response behavior, it was added that all help seekers would accept appointments at the times offered by the therapists. After reading the instructions, all participants were presented with all 12 case vignettes in a randomized order in a within-subject design. The randomization was carried out with the aim of excluding possible sequence effects. Upon reading each of the presented contact forms, the therapists were asked to rate the probability with which they would offer a therapy place, and to decide intuitively based on the available information.

The average duration of study participation was 16.29 min. After the data collection was completed, the participants were informed by email about the real purpose of the study in the form of a debriefing.

2.5 | Statistical analysis

The analyses were conducted using SPSS 25 (IBM Corporate, 2017) and R (R Core Team, 2013; version 3.6.1). The "WRS2" package (Mair & Wilcox, 2019) was used for robust two-factor mixed analysis of variances (ANOVAs). Missing values in the independent variables occurred for two participants regarding the parental SES of the therapist and for four participants regarding the social status of the city district in which the practice is located. These participants were excluded from the respective analyses, such that the sample sizes were reduced to $N = 502$ and $N = 500$, respectively. There were no missing values for the main variable (probability of offering a therapy place).

A $3 \times 2 \times 2$ repeated measures ANOVA for SES and the two other relevant case vignette variables diagnosis and gender was calculated to investigate potential main effects of SES, disorder and gender of the patient, and interaction effects between SES and gender, and SES and mental disorder, on the likelihood of a therapy place offer. The data were tested for normality and sphericity, that is, the equality of the variances of all possible difference variables (Eid et al., 2010). The Shapiro–Wilk test (Shapiro & Wilk, 1965) indicated that the dependent variable was not normally distributed in any of the groups. However, since $N \geq 30$ in all groups, a normal sampling distribution according to the central limit theorem can be assumed, which suggests robustness of the method to the violation of the assumption of normality (Eid et al., 2010). The sphericity assumption was assessed using Mauchly's test (Mauchly, 1940). In cases in which this led to a significant result, that is, in which sphericity was not given, the degrees of freedom were Greenhouse–Geisser corrected (Greenhouse & Geisser, 1959) and the corrected p value was interpreted. In addition to the F value and the p value, the partial η^2 is reported for all significant main effects.

According to Cohen (1988), $\eta^2 \approx 0.01$ corresponds to a small, $\eta^2 \approx 0.06$ to a medium, and $\eta^2 \approx 0.14$ to a large effect. Regarding the multiple pairwise comparisons (post-hoc tests), Bonferroni adjustment was applied to avoid α -error accumulation.

To assess interaction effects of the therapist's parental SES, type of therapy license, billing authorization with the statutory health insurance ("Kassensitz"), and social status of the urban district in which the practice was located, with the client's SES on the probability of a therapy place offer, we conducted multifactorial mixed ANOVAs. We assessed the data for normal distribution as well as sphericity and homoscedasticity, that is, the equality of the variances between the levels of the nonrepeated factor, and the equality of the variance-covariance matrices, which must also exist between the levels of the non-repeated measures factor (Eid et al., 2010). The violation of the assumption of normal distribution as indicated by a significant Shapiro-Wilk test (Shapiro & Wilk, 1965) was again unproblematic, since all groups were based on $N \geq 30$ and the procedure is robust to violations of normality (Eid et al., 2010). In contrast, violations of the homoscedasticity assumption assessed with the Levene test and Box's test were of relevance for the further statistical procedure (Eid et al., 2010). The Box-Cox transformation (Box & Cox, 1964) did not lead to any significant improvement in terms of homoscedasticity, and so-called robust mixed ANOVAs, which are robust to violations, had to be used (Eid et al., 2010; Mair & Wilcox, 2018). A disadvantage of robust mixed ANOVAs is that effect sizes cannot be determined. All multiple pairwise comparisons were calculated as Games-Howell post-hoc tests (Games & Howell, 1976), which should be used in the presence of heteroscedasticity. For all analyses, the significance level was set at $\alpha = 0.05$.

3 | RESULTS

3.1 | Description of the therapist sample

A total of 3183 therapists were contacted by email and invited to participate in the study, with the link to the online study being sent immediately. The email was deliverable in 2990 cases. A total of 785 therapists ultimately participated in the study, 504 of whom (64%) completed the survey in full. A description of the sample is provided in Table 1.

3.2 | Offer of therapy places

The participants used the entire Likert scale (range = 1–11). The average probability of a therapy place offer across all participants and case vignettes was 7.86 (SD = 2.65). Table 2 shows the results of all ANOVAs.

Potential clients' SES had a significant influence on the probability of a therapy place offer ($F(1, 503) = 115.64$; $p = 0.000$): Clients with a low SES were 4.1% less likely to be offered a therapy place, with a large effect size ($\eta^2 = 0.187$). The influence of SES on the probability of a therapy place offer also varied significantly depending on the patient's disorder ($p = 0.000$): SES had a greater effect for bulimia than for depression and social anxiety disorder. The influence of SES was least pronounced for social anxiety disorder. Overall, however, the effect was small ($\eta^2 = 0.026$). No statistically significant interaction between gender and SES was observed ($p = 0.062$).

With respect to the parental SES of the psychotherapist there was no variation in the influence of SES ($p = 0.664$).

For the therapy method for which the therapist was licensed a significant interaction with SES was observed ($p = 0.000$), insofar as SES had a greater effect on the probability of a therapy place offer when the therapist was a psychoanalyst as compared to psychodynamic and behavioral therapists. There was no significant interaction between billing authorization with the statutory health insurance and the client's SES ($p = 0.723$).

TABLE 1 Descriptive statistics therapist sample

Demographics	M	SD	Range
Age	51.60	11.45	28–85
	N	%	
Gender			
Male	124	25	
Female	379	75	
Family status			
Single	138	27	
Married	293	59	
Separated/divorced	64	13	
Widowed	6	01	
City			
Berlin	249	49	
Hamburg	117	23	
Cologne	79	16	
Bremen	36	07	
Leipzig	23	05	
Licence			
PA	72	14	
Depth psy.	149	30	
CBT	283	56	
Billing authorization			
Yes	358	71	
No	146	29	
Social status district			
Low	96	19	
Medium	129	26	
High	275	55	
Parental SES therapist			
Low	32	06	
Medium	291	58	
High	179	36	

Abbreviations: CBT, cognitive behavioral therapy; M, mean, PA, psychoanalysis; SD, standard deviation; SES, socioeconomic status.

TABLE 2 Analyses of variance (repeated measures and robust mixed ANOVAs) results on the likelihood of offering a therapy place

Independent variables	M	SD	F (1, 503)	Significance (p) of main effect	η^2
SES			115.64	0.000	0.187
Low	7.65	2.86			
Medium/high	8.06	2.68			
Independent variables	M	SD		Significance (p) of interaction effects	η^2
Disorder $\times$ SES				0.000	0.026
Social phobia $\times$ low	8.10	2.44			
Social phobia $\times$ medium/high	8.29	2.45			
Bulimia $\times$ low	6.56	3.06			
Bulimia $\times$ medium/high	7.13	2.83			
Depression $\times$ low	8.30	2.20			
Depression $\times$ medium/high	8.76	1.98			
Parental SES $\times$ SES				0.664	-
Low $\times$ low	6.83	2.07			
Low $\times$ medium/high	7.13	2.08			
Medium $\times$ low	7.70	2.22			
Medium $\times$ medium/high	8.10	2.06			
High $\times$ low	7.71	2.30			
High $\times$ medium/high	8.15	2.15			
Therapeutic method $\times$ SES				0.000	-
PA $\times$ low	6.39	2.17			
PA $\times$ medium/high	7.08	2.09			
Depth psy. $\times$ low	6.99	2.22			
Depth psy. $\times$ medium/high	7.47	2.17			
CBT $\times$ low	8.33	2.04			
CBT $\times$ medium/high	8.62	1.90			
Billing author $\times$ SES				0.723	-
Yes $\times$ low	7.38	2.25			
Yes $\times$ medium/high	7.75	2.13			
No $\times$ low	8.32	2.11			
No $\times$ medium/high	8.80	1.85			
Social status district $\times$ SES				0.000	-
Low $\times$ low	7.97	2.01			
Low $\times$ medium/high	8.08	2.01			

TABLE 2 (Continued)

Independent variables	M	SD	Significance (<i>p</i>) of interaction effects	η^2
Medium $\times$ low	7.43	2.39		
Medium $\times$ medium/high	7.95	2.23		
High $\times$ low	7.66	2.25		
High $\times$ medium/high	8.11	2.08		

Abbreviations: CBT, cognitive behavioral therapy; M, mean; n.s., not significant; PA, psychoanalysis; SD, standard deviation; SES, socioeconomic status.

A statistically significant interaction emerged between the social status of the city district in which the psychotherapist's practice was located and the client's SES ($p = 0.000$). In socially disadvantaged districts, the influence of SES on the probability of a therapy place offer was less pronounced (difference in the mean probability of a therapy place offer between medium/high and low SES = 0.11) than in medium or strong districts (difference in the mean probability of a therapy place offer between medium/high and low SES = 0.52 and 0.45, respectively).

All analyses were repeated without 76 participants who recognized the true purpose of the study. The results remained unchanged (see Supporting Information: Appendix 3).

4 | DISCUSSION

Overall, the results indicated that help seekers with a higher SES were 4.1% more likely to be offered a therapy place. This finding is in line with the results of previous studies from the US conducted by Kugelmass (2016, 2018). Given the considerable differences between the American and German health care systems, the current findings indicate health disparities in the access to treatment in Germany, as the costs for psychotherapy are fully covered by the statutory health insurances (Bijl et al., 2003). However, important is that the main effect and all secondary findings were relatively modest.

That is, in addition we found a significant variation in the influence of SES on the probability of access to treatment depending on the disorder. SES had a significantly greater impact on the likelihood of being offered a therapy place in the case of bulimia, compared to depression and social anxiety disorder. This might correspond to the assumption that the influence of SES might be stronger when a person has other less preferred characteristics, even though previous research did not report negative tendencies towards patients with bulimia in general (Zepf et al., 2003). Importantly, again the differences are modest. Nevertheless, to effectively counteract any such reservations, this issue could be investigated in greater depth in future studies. The influence of SES on access to treatment did not differ between male and female potential clients in the current study.

Our results indicate that the treatment method is also relevant for therapists' decision to offer treatment to clients with low SES. The influence of SES on treatment allocation by psychoanalysts and depth psychologists corresponds to the results of previous studies, which reported greater reservations towards patients with a low SES among psychodynamic (Sutton & Kessler, 1986) or psychoanalytic (Zepf et al., 2003) therapists. However, Saxon et al. (2007) found no influence of the patient's SES on the offer of psychoanalysis or behavioral therapy, or on the patient's acceptance by the respective therapist. Possibly, this might be explained by the fact that the study was carried out within a National Health Service psychotherapy facility in the United Kingdom. However, even though the findings were significant again the differences were small.

The social status of the city district in which the practice was located also caused a significant, but modest variation in the influence of SES on the likelihood of offering a therapy place. The impact of SES was less

pronounced for therapists whose practice was located in a socially disadvantaged urban district than for psychotherapists whose practice was based in a medium or strong urban district. This may be because therapists in socially weaker districts more frequently receive inquiries from help seekers with a low SES. However, it is also conceivable that therapists who locate their practice in socially weaker urban districts have fewer reservations about clients with low SES. The general preference for socially stronger city districts as the location of one's own practice is in line with the levels of health care provision of the individual Berlin districts, which vary significantly depending on the social status of the respective city district (Joint State Committee for Berlin [Gemeinsames Landesgremium für Berlin], 2018; Senate Department for Health and Social Affairs [Senatsverwaltung für Gesundheit und Soziales], 2013). For the other cities, no official data were available.

Regarding the influence of SES on the likelihood of offering a therapy place, there were no significant variations between psychotherapists with different parental SES in terms of the number of therapy places available. The past experience of therapists with a low parental SES does not seem to have led to fewer reservations towards clients with a low SES. However, these findings should be interpreted with caution, because only 6% of the therapists in our sample had a low parental SES. An early study by Sutton and Kessler (1986) also reported no significant interaction between the SES of the patient and the parental SES of the therapist. It is even possible that the social advancement of therapists achieved through education leads them to distance themselves from individuals with a low SES despite their similar origins (Lott, 2002). In view of the still existing social inequalities in the German education system, which make it more difficult for people with a lower parental SES to gain access to higher education and university studies, it is unsurprising that almost all psychotherapists in the sample had a medium or high parental SES (Heine, 2010). Moreover, the subsequent training to become a psychological psychotherapist entails a high financial burden, which can represent a further major obstacle for persons who do not have access to the appropriate resources (Strauß et al., 2009).

Taken together, the results of the current study indicate a disadvantage of help seekers with low SES in terms of the offer of therapy places, which is also affected by other characteristics of both the potential clients and the therapists, but all effects were relatively modest.

In general, it is assumed that a discrepancy between the high prevalence of mental disorders and the low utilization of outpatient treatment by individuals with low SES might be caused both by factors related to the individuals with low SES themselves, such as lower health literacy and less help seeking behavior, as discussed in the literature, and by biases of the therapists, of which the therapists themselves might not be aware. We consider it important to enhance the awareness of practitioners, and hope that the results of the current study encourage them to adopt an unbiased and appreciative attitude towards every (potential) client irrespective of SES. The explicit reflection on one's own motives is of great importance, as the following comment from one study participant illustrates: "I have noticed that I currently have 70% patients with university degrees or students, that was not so clear to me until now." Gathering experience can also prevent a feeling of lack of competence. This becomes clear in a comment by another study participant: "It was actually the case that I was very afraid of doing therapy with people from a modest background because I was afraid that I would not be able to make them a suitable offer. But that has not come true, I now find it very enriching to have contact with people from so many different backgrounds and classes." Awareness of the relevance of SES could be part of psychotherapy training, which is in line with a recommendation given by the APA Task Force on Socioeconomic Status (American Psychological Association [APA], Task Force on Socioeconomic Status, 2007). This is important because disadvantaging persons with a low SES in terms of therapy place offers contradicts the welfare state requirement, according to which both the state and psychotherapists are obliged to care for persons with statutory health insurance (Simon, 2013). Moreover, it also violates the professional regulations of the state chambers of psychotherapists (e.g., Chamber of Psychotherapists Berlin [Psychotherapeutenkammer], 2016; BPtK [Federal Chamber of Psychotherapists/Bundespsychotherapeutenkammer], 2018; East German Chamber of Psychotherapists [Ostdeutsche Psychotherapeutenkammer], 2014).

Moreover, the topic could also be already integrated into the curriculum for university students specializing in clinical and counseling psychology. In addition, it is important to admit students from various SES backgrounds who

will bring these issues to light due to their differing perspectives, and to address the retention of lower SES students and students from minority groups even after they have been admitted to courses and qualified. Ensuring diversity of students and therapists is important so that they match the general population profile and can better meet the needs also of minorities in society (Psychology Workforce Task Force, 2016). Upward social mobility in general is rare in Germany, and also other countries such as in the US, and social inequality in academia has recently come into focus due to diversity movements, issues of institutional discrimination, and attempts to structurally increase equality in universities in general (Browman et al., 2017; Lenger, 2008; Moeller, 2013; Shane & Heckhausen, 2017).

5 | LIMITATIONS

Like any study, the current investigation is subject to some limitations. First, a convenience sample was investigated. It was not possible to verify the representativeness of the sample regarding central aspects, as the relevant data were not available from the German Chamber of Psychotherapists. Second, only three disorders were included in the current study. Therefore, future studies should also investigate the relationship between SES and the offer of therapy places for other disorders. Third, case vignettes were used as stimuli. There are concerns that vignettes are inadequate simplifications of a complex reality (Evans et al., 2015). Regarding the current study, it should be noted that in their everyday work, therapists indeed have much more information about clients on which to base their decisions, such as external appearance and interaction behavior. Fourth, even though we conducted a pilot study and power analysis to increase the validity and reliability of the measurement and to determine the required sample size, the within-subject design might have decreased the experimental validity, as it required that the vignettes differed from one another on variables other than SES (Atzmüller et al., 2016; Westfall et al., 2014). However, in addition to the factors that were systematically varied (SES, disorder and gender), only the symptoms as well as the age differed. We tried to reduce the impact on the experimental validity by keeping the number of symptoms and the age range constant in all vignettes.

A variation of the method would be of particular interest and a field study with simulated voicemail messages based on Kugelmass (2016) could be informative. The same applies to the implementation of a field study with fictitious e-mails, as also carried out by Kugelmass (2018). However, numerous studies have shown that the answers given in vignette studies are very close to those in real-life situations (Evans et al., 2015). Fourth, participation in the study was restricted to therapists working in large cities, and the results cannot be transferred to conditions in rural areas. On the one hand, due to the poorer health care situation in rural areas (BPtK [Federal Chamber of Psychotherapists/Bundespsychotherapeutenkammer], 2018), it is conceivable that the effect of clients' characteristics on therapy offers is larger in rural areas, as the therapists have to be even stricter in their selection. On the other hand, the influence of SES might be lower, since rural areas are often structurally weak regions and therapists might be more accustomed to working with clients with a low SES. Therefore, an investigation of regional differences in the influence of the SES on the offer of therapy places would be another interesting aspect for future research.

6 | CONCLUSION

Therapist biases contributing to unequal access to treatment for individuals with low SES and other characteristics can aggravate mental health care disparities. The current state of research on the criteria by which therapists allocate their therapy places and on the relevance of SES on this decision is rather poor, and there is an urgent need for future studies that explicitly address this topic. Raising awareness of the association between SES and the offer of therapy places might help practitioners to reflect on their own selection practices and to reduce mental health care disparities.

AUTHOR CONTRIBUTIONS

Helen Niemeyer and Christine Knaevelsrud designed the study. Helen Niemeyer conducted the study and wrote the first draft of the manuscript. Both authors contributed to and have approved the final manuscript.

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CONFLICT OF INTEREST

The authors declare no conflict of interest.

DATA AVAILABILITY STATEMENT

The data that support the findings of this study are openly available in OSF at <https://osf.io/z34s2/>.

TRANSPARENT PEER REVIEW

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ORCID

Helen Niemeyer  <http://orcid.org/0000-0002-7616-1885>

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SUPPORTING INFORMATION

Additional supporting information can be found online in the Supporting Information section at the end of this article.

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